NCE EXAM PREP

Practice Exam 29

Domain 5

Indirect Client Care

21 Practice Questions | 28 minutes

A, B, C answer choices

Answer Key and Explanations included

> **Instructions.** Select the single best answer for each question. Record your answers separately. This domain practice set is drawn from Full-Length Practice Exam 05. Use the suggested 28-minute practice timer. Do not consult Part B until you complete Part A. These domain timings are editorial practice allocations, not official NBCC section limits.

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PART A — QUESTIONS

Choose one answer (A, B, or C) per question.

Question 1

> During a session, Aiden says, "I have panic all the time." When the counselor asks for detail, Aiden reports three discrete panic episodes during the past week and ongoing worry between episodes.

Which documentation is most accurate?

A) Aiden has constant severe panic disorder with no symptom-free periods.

B) Aiden described panic as occurring "all the time," then reported three episodes in the past week and worry between episodes.

C) Aiden experienced three panic episodes, with no need to document the continuing worry or his initial description.

Question 2

> Riley has authorized both a sibling and a peer specialist to attend a support-planning meeting. They repeatedly interrupt one another, and Riley becomes quiet. Riley tells the counselor during a break that both perspectives matter but that Riley wants to make the final decisions.

How should the counselor resume the meeting?

A) Establish client-approved turn-taking and role boundaries, then direct decisions back to Riley

B) Ask the sibling to decide because family members know Riley outside service settings

C) Remove both supporters and complete the plan with Riley without discussing other meeting options

Question 3

> Evan tells his counselor that a medication copayment doubled and that he has begun taking the medication less often to make it last. He reports no current symptoms. A written authorization permits coordination with the prescriber and pharmacist.

What should the counselor do?

A) Document Evan's exact report and help him promptly explore prescriber- or pharmacist-approved affordable options and legitimate assistance

B) Recommend taking the medication every other day until Evan can afford a refill

C) Tell Evan to replace the medication with a lower-cost nonprescription product

Question 4

> Leah brings a bottle of medication that her prescriber discontinued and asks the counselor to dispose of the remaining tablets. A nearby pharmacy participates in a verified medication take-back program.

What should the counselor do?

A) Flush the tablets so Leah does not accidentally take them

B) Store the bottle in the counseling office until several clients have medication to discard

C) Decline to take possession and direct Leah to the verified pharmacy take-back process

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Question 5

Which action most clearly distinguishes care coordination from simply giving a referral?

A) Give the client several telephone numbers and place responsibility for all follow-up on the client

B) Contact providers without client authorization so services can begin more quickly

C) With client authorization, align relevant roles and information and establish follow-up around the client's goal

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Question 6

Which practice best reduces ambiguity from abbreviations in clinical documentation?

A) Shorten every recurring phrase so notes can be completed as quickly as possible

B) Create personal shorthand and rely on coworkers to infer its meaning from context

C) Use only approved, commonly understood abbreviations and define any term that could have more than one meaning

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Question 7

Which feature makes a referral list more resilient during service disruptions?

A) Dependence on one preferred provider for each need so counselors always know whom to call

B) Multiple independently verified pathways, including practical backup options and access instructions

C) A large national list added without local verification or information about how clients enter services

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Question 8

> A counselor discusses an evening group and individual counseling with Maribel. Maribel chooses individual counseling because group time conflicts with childcare, and the counselor schedules the next individual appointment.

What should the note record?

A) The options discussed, Maribel's selection and stated reason, and the agreed next appointment

B) That Maribel refused group treatment and is resistant to the recommended level of care

C) Only the individual appointment, because her response to the other option is not clinically relevant

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Question 9

> Owen has authorized coordination with a childcare-subsidy agency. The assigned liaison has not responded to messages for 10 days, and the application deadline is in three days. Owen wants to continue the application but does not want the counselor to submit information in his name.

What should the counselor do?

A) Keep waiting for the assigned liaison because contacting anyone else would disrupt the agency's process

B) Submit the application for Owen so the deadline is not missed

C) With Owen, use a verified alternate agency contact or escalation route and update the coordination plan

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Question 10

> At a planning meeting, a job coach describes the client as unmotivated after a missed interview. The client says the employer changed the interview time without notice and offers to show the message. The counselor is authorized to coordinate with both parties, and the team is considering changing the employment plan.

What should the counselor facilitate?

A) Accept the coach's interpretation because the coach manages the employment service

B) Record the differing accounts and identify what information should be reviewed before the team changes the plan

C) Exclude the coach's perspective because the client's account should never be questioned

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Question 11

> A qualified interpreter assists during a session in which the client reports insomnia. The counselor's draft note says, "The interpreter reported that the client cannot sleep." The counselor heard the interpretation while speaking directly with the client.

How should the note be revised?

A) Treat the interpreter as the clinical informant because the counselor did not understand the client's original words

B) Omit the sleep concern because it was communicated through another person

C) Identify the language support used and attribute the insomnia report to the client rather than the interpreter

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Question 12

> After a refill, Marcus notices that his prescribed tablets have a different color and shape. The bottle lists the same medication name and dose, but Marcus asks his counselor whether the tablets are correct. He reports no symptoms, and an authorization permits contact with the pharmacy.

What should the counselor do?

A) Assure Marcus that appearance never matters when the medication name on the bottle is unchanged

B) Tell Marcus to stop the medication until his next counseling appointment

C) Document the concern and help Marcus obtain verification from the dispensing pharmacist

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Question 13

Which review process would most improve a counseling program's referral resources over time?

A) Examine de-identified patterns of successful and failed linkages and recurring barriers, then update resource vetting accordingly

B) Count how many resource brochures counselors distribute without checking whether clients connect

C) Remove any program with low use before determining whether the cause is poor fit, limited awareness, or an access barrier

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Question 14

> Kayla has authorized coordination with a housing advocate, college advisor, benefits counselor, and family support person. She tells her counselor that meetings with all four people at once are overwhelming and that she stops asking questions. Several upcoming decisions require only one or two of the stakeholders.

How should the counselor organize future coordination?

A) Ask Kayla which decisions and participants belong together and use smaller meetings when they can accomplish the goal

B) Continue inviting everyone because a signed authorization means all stakeholders should attend each meeting

C) Hold provider-only meetings so Kayla does not have to manage the stress of participating

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Question 15

> Mei speaks Cantonese and prefers written materials in Traditional Chinese. The clinic's current handout is available only in Simplified Chinese, which Mei says she cannot read comfortably. A qualified translation service is available.

How should the counselor provide the material?

A) Use the Simplified Chinese handout because spoken Cantonese means either written form should be adequate

B) Obtain qualified translation in Mei's preferred written form and confirm that the resulting support is usable for her

C) Ask a family member to rewrite the handout informally in whichever characters seem familiar

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Question 16

A client leaves a scheduled counseling session 20 minutes early because a childcare provider calls with an urgent problem. The counselor and client partially complete the planned topic and schedule follow-up. The client shows no acute clinical concern.

How should the counselor document the encounter?

A) Record the actual duration, work completed, reason for ending early as the client reported it, and the follow-up plan

B) Record the full scheduled duration because the early departure was unrelated to counseling

C) Describe the client as noncompliant and omit the partially completed work

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Question 17

Ana is a seasonal agricultural worker who moves between two regions. She authorizes community health workers and counselors in both locations to coordinate. Each move has caused pending transportation, benefits, and appointment tasks to be restarted because the next team cannot tell what was completed.

What would best support continuity across locations?

A) Ask Ana to retell the entire history whenever she arrives in the other region

B) Create a client-approved portable action summary with current contacts, completed steps, pending tasks, and a confirmed handoff

C) Have both teams work on every task simultaneously so nothing is missed

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Question 18

A client reports nightmares and avoidance after a frightening event. The counselor has not yet established symptom duration, functional impact, or the other information needed to determine whether a trauma-related diagnosis is supported.

How should the counselor document the current assessment?

A) Confirm a trauma-related diagnosis because nightmares and avoidance are sufficient

B) Record the reported symptoms, the current clinical considerations, the missing information, and the plan for further assessment

C) Omit the symptoms until enough information exists to assign a diagnosis

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Question 19

A counselor's draft note states that a client has shown "50% improvement" because the client's current anxiety score is 8. The earlier record describes anxiety as moderate but contains no baseline numerical score from the same measure.

How should the counselor revise the note?

A) Keep the percentage because a current score of 8 is consistent with substantial improvement

B) Record the current score and the available qualitative comparison without claiming an unsupported percentage

C) Estimate a baseline score from the word "moderate" and use it to calculate the percentage

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Question 20

A community support team posts schedules and task updates in a project-management application. The client has only a prepaid telephone with text messaging and cannot open the application reliably. The client has authorized team communication but repeatedly receives updates after decisions have been made.

What should the counselor recommend?

A) Require the client to obtain a smartphone so the team can keep one communication system

B) Let the providers continue using the application and ask one member to summarize decisions afterward

C) Establish a client-accessible update method and keep it synchronized with the team's internal system

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Question 21

Farah requests a woman interpreter for a trauma-focused intake. A qualified woman interpreter is available the next day, while a qualified man is available immediately. Farah reports no urgent concern and says she is comfortable waiting for the woman interpreter.

What is the best response?

A) Schedule the qualified woman interpreter, or discuss other acceptable qualified options if Farah's needs change

B) Use the male interpreter immediately because professional qualification makes client preference irrelevant

C) Ask Farah to bring a woman friend who can interpret instead of delaying the intake

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PART B — ANSWER KEY AND EXPLANATIONS

Answers match the numbered items in Part A.

Question 1 — Correct: B

Correct Answer: Aiden described panic as occurring "all the time," then reported three episodes in the past week and worry between episodes.

Explanation: This entry preserves Aiden's language and the more specific frequency and time frame obtained through clarification. It does not convert colloquial wording into a diagnosis or a claim of continuous panic. Omitting the between-episode worry would also discard relevant information.

Question 2 — Correct: A

Correct Answer: Establish client-approved turn-taking and role boundaries, then direct decisions back to Riley

Explanation: A structured meeting can retain both supports while restoring Riley's participation and decision-making role. Riley has said both perspectives matter, so immediate exclusion would not reflect the stated preference. Giving the sibling control would also displace Riley's authority.

Question 3 — Correct: A

Correct Answer: Document Evan's exact report and help him promptly explore prescriber- or pharmacist-approved affordable options and legitimate assistance

Explanation: The cost barrier is already changing how Evan takes the medication and warrants prompt coordination with responsible professionals. They can identify an approved alternative, assistance pathway, or other solution. The counselor should not design a reduced dosing schedule or substitute a nonprescription product.

Question 4 — Correct: C

Correct Answer: Decline to take possession and direct Leah to the verified pharmacy take-back process

Explanation: A verified take-back program provides an appropriate disposal route without making the counselor responsible for storing or destroying medication. Flushing may be inappropriate, and accumulating medication in the office introduces avoidable handling and storage concerns. The counselor can support the linkage while leaving disposal to the designated program.

Question 5 — Correct: C

Correct Answer: With client authorization, align relevant roles and information and establish follow-up around the client's goal

Explanation: Coordination adds purposeful communication, role alignment, and follow-through to the referral process. It remains centered on the client's goal and authorization. A list of telephone numbers alone does not create a coordinated plan.

Question 6 — Correct: C

Correct Answer: Use only approved, commonly understood abbreviations and define any term that could have more than one meaning

Explanation: Standardized and unambiguous language allows future readers to interpret the note consistently. Personal shorthand creates avoidable uncertainty, even when coworkers can sometimes infer its meaning. Efficiency does not justify reducing documentation clarity.

Question 7 — Correct: B

Correct Answer: Multiple independently verified pathways, including practical backup options and access instructions

Explanation: Service disruptions can close or overload a single pathway, so verified alternatives make the directory more usable. Access instructions help counselors and clients shift routes efficiently. A large unverified list may look comprehensive while offering little reliable access.

Question 8 — Correct: A

Correct Answer: The options discussed, Maribel's selection and stated reason, and the agreed next appointment

Explanation: The note should represent an informed choice shaped by a concrete scheduling constraint. Maribel accepted counseling rather than rejecting care, so labeling her resistant would be inaccurate. Recording the alternatives and reason helps later readers understand the plan.

Question 9 — Correct: C

Correct Answer: With Owen, use a verified alternate agency contact or escalation route and update the coordination plan

Explanation: The approaching deadline and prolonged nonresponse justify using a verified alternative pathway while keeping Owen involved. This preserves his decision not to have the counselor submit information in his name. Waiting passively could allow an avoidable deadline failure.

Question 10 — Correct: B

Correct Answer: Record the differing accounts and identify what information should be reviewed before the team changes the plan

Explanation: The meeting contains a disputed interpretation and a potentially verifiable scheduling message. Recording both accounts and checking relevant information supports a more accurate employment plan. Automatically accepting or excluding either perspective would replace inquiry with an unsupported conclusion.

Question 11 — Correct: C

Correct Answer: Identify the language support used and attribute the insomnia report to the client rather than the interpreter

Explanation: The interpreter conveyed the client's statement but did not become the source of the symptom report. Documenting the language support explains how communication occurred while retaining correct attribution. Omitting interpreted content would erase clinically relevant information obtained during the session.

Question 12 — Correct: C

Correct Answer: Document the concern and help Marcus obtain verification from the dispensing pharmacist

Explanation: A change in appearance may have a routine explanation, but the pharmacist is the appropriate professional to verify what was dispensed. The counselor can support that contact without declaring the tablets correct or directing discontinuation. Marcus's lack of symptoms does not resolve the identification question.

Question 13 — Correct: A

Correct Answer: Examine de-identified patterns of successful and failed linkages and recurring barriers, then update resource vetting accordingly

Explanation: Connection outcomes and barrier patterns reveal whether listed resources function in practice and where directory information needs improvement. Brochure counts measure distribution rather than access. Low use should be understood before a potentially valuable resource is removed.

Question 14 — Correct: A

Correct Answer: Ask Kayla which decisions and participants belong together and use smaller meetings when they can accomplish the goal

Explanation: Authorization permits participation but does not require every stakeholder to attend every interaction. Staging focused meetings can preserve Kayla's voice and reduce unnecessary burden while still including relevant expertise. Provider-only meetings would remove her from decisions rather than address the meeting design.

Question 15 — Correct: B

Correct Answer: Obtain qualified translation in Mei's preferred written form and confirm that the resulting support is usable for her

Explanation: Spoken language and preferred written form are separate access considerations. Qualified translation into Traditional Chinese responds to Mei's stated reading need, and confirmation checks whether the support works in practice. A family rewrite would not provide equivalent translation quality.

Question 16 — Correct: A

Correct Answer: Record the actual duration, work completed, reason for ending early as the client reported it, and the follow-up plan

Explanation: Accurate documentation reflects the service actually delivered and the known reason for its early end. It also preserves the partially completed work and continuity plan. Recording the scheduled duration or labeling noncompliance would misrepresent the encounter.

Question 17 — Correct: B

Correct Answer: Create a client-approved portable action summary with current contacts, completed steps, pending tasks, and a confirmed handoff

Explanation: A concise action summary can move with Ana and prevent tasks from restarting or disappearing at each transition. Confirmation with the receiving team closes the handoff while keeping Ana informed. Duplicating every task across both regions would create confusion and wasted effort.

Question 18 — Correct: B

Correct Answer: Record the reported symptoms, the current clinical considerations, the missing information, and the plan for further assessment

Explanation: The symptoms are clinically relevant even though the available information does not establish a diagnosis. Documenting what is known, what remains uncertain, and how it will be assessed preserves accurate reasoning. Confirming a diagnosis now would exceed the evidence.

Question 19 — Correct: B

Correct Answer: Record the current score and the available qualitative comparison without claiming an unsupported percentage

Explanation: A percentage change requires comparable numerical data at both points. The earlier qualitative description can be documented, but it cannot supply a precise baseline score. Estimating a number would create false measurement precision.

Question 20 — Correct: C

Correct Answer: Establish a client-accessible update method and keep it synchronized with the team's internal system

Explanation: Coordination should not exclude the client from timely information simply because providers prefer a particular application. A synchronized text or other accessible pathway can preserve both team efficiency and client participation. Summaries delivered only after decisions are made do not solve the access problem.

Question 21 — Correct: A

Correct Answer: Schedule the qualified woman interpreter, or discuss other acceptable qualified options if Farah's needs change

Explanation: Farah has expressed a communication preference relevant to her ability to participate in a trauma-focused intake. A qualified interpreter who meets that preference is available without creating an urgent delay. A friend would not be an equivalent substitute for qualified language support.
